

# THE FIELD GUIDE *to* the School-Based Ecosystem<sup>©</sup>

Written by: Topher Fuqua, LMFT



Name: \_\_\_\_\_

Start Date: \_\_\_\_\_

Assigned School(s): \_\_\_\_\_

Supervisor Name: \_\_\_\_\_



Hi. And welcome.

Take a breath.

This work isn't going to be easy. But it is meaningful work, more that you can imagine as you first get started. Both of those things will be true on the same Tuesday afternoon — probably between a no-show and a fire drill.

This workbook isn't a test. It's not a performance tracker. It's not going to judge you for the sessions that went sideways or the notes you wrote late. It's a companion — something you can scribble in, dog-ear, spill coffee on, and come back to when the week gets loud. Think of it as a field notebook for the most complex, beautiful, chaotic terrain you'll ever work in.

You don't have to have it figured out. Not today. Not by month three. Not even by the end of this year. You just have to be willing to show up, stay curious, and let the ecosystem teach you. The school building is going to surprise you. The students are going to surprise you. You're going to surprise yourself.

This guide was created by someone that started right where you are sitting, who walked into a school building for the first time and thought, *"What did I get myself into?"* Who couldn't find their designated workspace. Who was so shy they spent their first week hiding in their office decorating. Who had a session plan fall apart in the first four minutes and had to improvise with a deck of cards and a prayer. You're in good company.

I built this guide because I wanted you to have something I didn't — a map, a mirror, and a gentle reminder that the learning curve isn't a flaw. It's the path.

So, here's what I ask: Use this field guide. Write in the margins. Fold the corners. Bring it to supervision. Let it collect the evidence that you are, in fact, figuring this out — one week, one session, one hard conversation at a time.

**Welcome to the field. Let's get to work.**

# Welcome to the Ecosystem

## Your Role in the Ecosystem



### Field ID Badge

My name in the building: \_\_\_\_\_

My species (clinical title): \_\_\_\_\_

My territory (office/room #): \_\_\_\_\_

I was trained to: \_\_\_\_\_

In this building, I am here to: \_\_\_\_\_

One thing I want staff to understand about my role:

One thing I'll remind myself when I forget my purpose:

#### I AM:

A clinical professional  
embedded in an educational  
ecosystem.

#### I AM NOT:

A substitute teacher,  
a disciplinarian,  
or a behavior aide.



# Section 1: Welcome to the Ecosystem

*"Every ecosystem has a rhythm. Your job is to learn it, acclimate to it, and be a consistent support."*

## 1.1 — Who You Are in the Building

You are not school staff. You are not an outsider. You are a clinical professional embedded in an educational ecosystem. That's a unique and sometimes confusing position — like being a marine biologist on a fishing boat. Everyone's looking at the same water, but you're seeing different things.

This section helps you understand your role — what it is, what it isn't, and why clarity here is the foundation everything else is built on. When you know who you are in the building, you stop apologizing for the space you take up. And you should absolutely stop apologizing.

### Role Clarity Quick Reference

| WHAT YOU ARE                                                                       | WHAT YOU ARE NOT                                                                   |
|------------------------------------------------------------------------------------|------------------------------------------------------------------------------------|
| A licensed/credentialed clinician providing therapeutic services                   | A substitute teacher, a disciplinarian, or a behavior aide                         |
| A collaborative partner with school staff                                          | A school employee who answers to the principal (though you respect their building) |
| An advocate for the students on your caseload                                      | A crisis hotline or an on-call resource for every student concern                  |
| A guest in the building who earns their place through presence and respect         | Responsible for fixing everything that's broken in a child's life                  |
| Part of a larger agency team with its own structure, supervision, and expectations | Available 24/7 — you have boundaries, and they matter                              |

## 1.2 — The Ecosystem Map

The school building is a living ecosystem with its own food chain, migration patterns, and weather systems. Here's who lives here — and where you fit. Consider this your species guide for the first few weeks.

### THE FRONT OFFICE STAFF

*"The gatekeepers. They know everything. Befriend them early and sincerely. They will make or break your daily experience. Learn their names. Ask about their weekends. Bring donuts at least once. These humans are the nervous system of the entire building."*

### THE TEACHERS

*"They see your students more than anyone. They're also exhausted. Approach with curiosity, not correction. They have data you need. You have insight they need. The magic happens when you build trust, not when you show up with a clipboard and suggestions."*

### THE ADMINISTRATORS

*"They run the building. Respect the chain of command. Keep them informed when appropriate. Don't surprise them. An administrator who trusts you will give you room to work. An administrator who's been blindsided will make your life complicated."*

### THE SUPPORT STAFF (Counselors, Social Workers, SLPs, etc.)

*"Natural allies. Collaborate early. Share resources. Avoid turf wars. You're not in competition; you're in an ecosystem. There's enough work for everyone, and the students benefit when the adults coordinate."*

### THE FAMILIES

*"Your clinical partners. Not your opponents. Not your audience. Partners. Some will be grateful. Some will be skeptical. Some won't return your calls. Meet every one of them where they are."*

### THE STUDENTS

*"The reason you're here. They didn't ask for a therapist. Earn their trust. It takes longer than you think. And when it clicks? That's the moment that makes all of this worth it."*

### CENTERSTONE TEAM

*"Your home base. Your supervisor, your fellow clinicians, your program manager. This is where you bring the hard stuff — the cases that keep you up at night, the questions you're afraid to ask, the wins that deserve celebration."*

# Who's in Your Habitat?

## Know Your School Ecosystem

### THE FRONT OFFICE STAFF

The Gatekeepers.



They know everything. Befriend them early.  
They'll make or break your day.

### THE TEACHERS

The Pack.



Loyal & tired. Build trust.  
They have the data. You have the insight.

### THE ADMINISTRATORS

The Bosses.



Respect the chain. No surprises.  
Trust gets you room to work.

### THE SUPPORT STAFF

The Allies.



Collaborate & share. Avoid turf wars.  
We're in this together.

### Notes to My Parent.



# Who's in Your Habitat?

More Key Players in Your School Ecosystem

## THE JANITORIAL STAFF

The Clean-Up Crew.



Hardworking & unseen.  
They keep everything running smoothly.

## THE LIBRARIAN

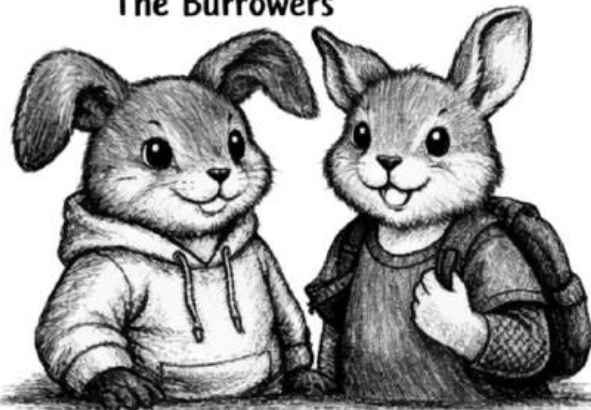
The Bookworm



Quiet & wise. The keeper of knowledge.  
Always there to help you find what need.

## THE STUDENTS

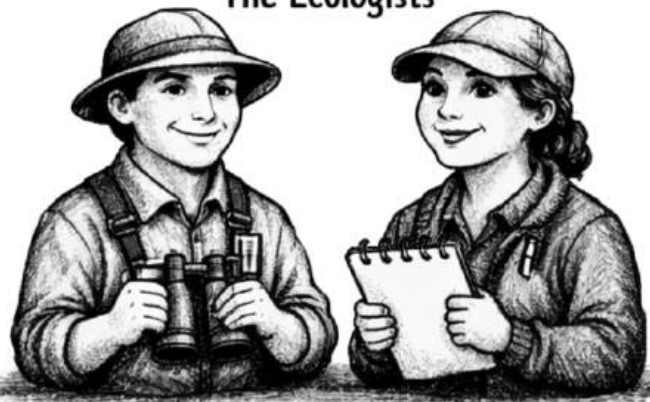
The Burrowers



Cautious & curious.  
Earn their trust. They're why you're here.

## THE CENTERSTONE TEAM

The Ecologists



Observers & guides.  
Studying the habitat.  
Supporting you every step of the way.

## Observing the Culture

Tune into the school's emotional atmosphere. Pay attention to how it feels around here. Be a climate reader!



### Observe the Mood

Are people stressed?  
Anxious? Hopeful? Welcoming?



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### Listen Closely

What's the common language?  
What's a whispered concern?



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### Watch the Hallways

What's the pace? Busy? Tense?  
Relaxed?



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### Immerse Yourself

Let people see you.  
Be present & involved.



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## 1.3 Expedition Inventory

Before you step into the field, let's take stock. Every researcher carries equipment — but also assumptions, hopes, fears, and habits. Some of what you're carrying will serve you well. Some of it might need to be set down. This isn't a judgment exercise. It's an awareness exercise. Grab a pen. Be honest. Nobody's grading this.

### Expedition Inventory Worksheet

Skills and strengths I'm bringing into this role:

Assumptions I have about school-based work:

Things I'm nervous about (be honest — this page is just for you):

What I hope this year teaches me:

Something I might need grace for myself in is:

# Trail Guide to Thrive

## Survival Tips for School-Based Clinicians



### Set Up Camp

3 things that make my office mine: \_\_\_\_\_



### Learn the Trails

Draw your daily route through the building: \_\_\_\_\_



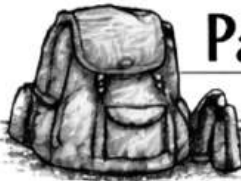
### Find Your Water Source

My water sources are: \_\_\_\_\_



### Build Your Fire

This week I'll connect with \_\_\_\_\_ by \_\_\_\_\_



### Pack Light

3 things I can let go of this week: \_\_\_\_\_



### Read the Weather

The energy in my building right now feels like: \_\_\_\_\_



# Section 2: Mapping Your Territory

*"A good map doesn't tell you where to go. It shows you what's possible — and where the drop-offs are."*

## 2.1 — The Master Schedule Blueprint

Your master schedule is the skeleton of your week. It's not a wish list — it's a structural commitment. You'll build it once, copy it weekly, and adjust it as you learn your ecosystem's rhythms. A good schedule doesn't just organize your time. It protects it. Here's how to build one that actually holds up.

### Step-by-Step Schedule Construction

1. **Block your non-negotiables first.** These are team meetings, supervision, and any standing agency commitments. These don't move, they aren't optional, they require participation. They are the bedrock. Build everything else around them.
2. **Map your school hours.** What time can you be in the building? What time does the school day start and end? When are students available for pull-out? Some buildings have strict windows — learn them before you schedule over them.
3. **Layer in your sessions.** Use the school's specials schedule (art, music, PE, library) to identify the best pull-out windows. Avoid pulling from core instruction — teachers will remember. And they will tell other teachers.
4. **Build in travel time.** If you serve multiple schools, be honest about drive time. Don't schedule back-to-back sessions across buildings. Traffic is not a clinical intervention. Arriving flustered is not a therapeutic stance.
5. **Protect your administrative windows.** These are not "open" time. These are scheduled, sacred blocks where you complete your documentation, trainings, session planning, and more. Put them on your schedule like client appointments. More on this later.
6. **Leave margin.** Not every hour needs to be filled. White space is where you absorb crises, handle unexpected parent calls, or just breathe. A schedule without margin is a schedule that breaks by Wednesday.

#### Common Scheduling Pitfalls

- Stacking all your sessions back-to-back and wondering why you can't think by 2pm
- Treating admin blocks as optional — then drowning in late notes by Thursday
- Saying yes to every teacher request for "just a quick check-in" (it's never quick)
- Not accounting for school events that will eat your schedule (testing weeks, field trips, assemblies, picture day, that surprise pep rally nobody warned you about)
- Burnout secondary trauma, and compassion fatigue directly impair clinical judgement, emotional attunement, and decision making under pressure. Make time for self-care, it helps preserve your competence.

## 2.2 — Reading the School Calendar

The school calendar is your weather forecast. It tells you when storms are coming (testing season, holidays, end-of-quarter), when things slow down (early release days, teacher workdays), and when your schedule will be disrupted no matter how carefully you planned. Study it early. Mark the dates that will affect your caseload. Adjust proactively, not reactively. The therapists who get ambushed by testing week every single year are the ones who never looked at the calendar in August.

## 2.3 — The Weekly Planning Template

This is your weekly cockpit. Photocopy it. Print it. Tape it to your clipboard. Use it every single week. It won't make your week perfect — but it will make it intentional.

### Weekly Planning Template

Week of:

School(s):

Supervision date/time:

Team meeting date/time:

### Top 3 This Week

*The three most important things to accomplish this week. Everything else is secondary.*

1. Client care! (**Always #1**)

2.

3.

# Reading the School Calendar

Be Prepared for the Forecast!



The school calendar is your weather forecast.  
It tells you when storms are coming —  
and when the skies will clear. Plan  
proactively, not reactively!



## Key Dates to Flag

- ☐ Testing windows (standardized, benchmarks, etc.)
- ☐ Report card periods / grading deadlines \_\_\_\_\_
- ☐ School breaks and holidays \_\_\_\_\_
- ☐ Early release / half days \_\_\_\_\_
- ☐ Field trips and assemblies \_\_\_\_\_
- ☐ Parent-teacher conference weeks \_\_\_\_\_
- ☐ IEP / 504 meeting seasons \_\_\_\_\_
- ☐ Spirit weeks / special events \_\_\_\_\_
- ☐ First and last weeks of each semester \_\_\_\_\_
- ☐ Centerstone deadlines & documentation due dates \_\_\_\_\_

### Storms on the Horizon:

Upcoming disruptions to prepare for:

- ☐
- ☐
- ☐

### Fair Skies Ahead:

Opportunities to look forward to:

- ☐
- ☐
- ☐





### Students to See This Week

| Student Initials | Day / Time | Session Type | Notes |
|------------------|------------|--------------|-------|
|                  |            |              |       |
|                  |            |              |       |
|                  |            |              |       |
|                  |            |              |       |
|                  |            |              |       |
|                  |            |              |       |
|                  |            |              |       |
|                  |            |              |       |
|                  |            |              |       |

*\* Prepare to be flexible with your schedule, always have a backup plan.*

Weekly Schedule Grid

| Block     | Mon | Tues | Wed | Thurs | Fri |
|-----------|-----|------|-----|-------|-----|
| Morning   |     |      |     |       |     |
| Midday    |     |      |     |       |     |
| Afternoon |     |      |     |       |     |

Carry Forward

What didn't get done this week that needs to move to next week?

Weekly Wins

Name at least one thing that went well. You need this more than you think.

# Mapping Your Territory

Map Your Expedition Through the School Landscape



## Plan Your Expedition: Map the Path

What fixed landmarks will guide you?

Morning announcements, time to start a group, lunchtime session with the student that dreads the cafeteria?



## Navigate the Terrain: Being Where You Need to Be

What terrain might slow you down?

☐ Conflicts & roadblocks to plan for: \_\_\_\_\_



☐ Backup times or locations for adjusted routes: \_\_\_\_\_



## Identify Your Tasks: What's On My Plate?

Client care is always #1, but what else needs to get done?



# Section 3: The Energy Cycle

*"Every ecosystem runs on energy. So do you. This section is about understanding where your professional energy comes from, where it goes, and how to keep the cycle sustainable."*

## 3.1 — How Targets Work (The Honest Version)

Let's talk about the thing nobody loves talking about: productivity. Targets . Billable hours. Here's the truth — understanding how this works isn't selling out. It's understanding the engine that keeps your position funded so you can keep doing this work. The therapy doesn't happen without the funding. The funding doesn't happen without the documentation. You don't have to love it. You do have to understand it.

### WHAT COUNTS AS A BILLABLE UNIT

- Direct client sessions (individual, family, group)
- Clinical assessments and intakes
- Collateral contacts/ treatment planning and review (with guardians when clinically documented)
- Crisis intervention (when it meets documentation threshold)

### WHAT DOES NOT COUNT (but still matters)

- Travel between schools
- Team meetings and clinical supervision
- Completing documentation (the note isn't the billable event — the session is)
- Emails, phone tag, coordination that doesn't rise to a collateral contact
- Setting up your space, organizing materials, waiting for students to be pulled
- School meetings, collaborating with teachers
- Trainings
- Session planning

### THE REFRAME:

Productivity isn't about being a billing machine.

It's about **sustainable service delivery**. If your numbers are consistently low, it's usually a scheduling or systems issue, not a character flaw. Maybe your pull-out windows are too narrow. Maybe you're spending forty minutes tracking down a kid who's at a birthday party in the cafeteria.

Chat with your Team Lead and don't be afraid to ask questions.

We want you to be set up for success when you complete your ramp up period.

## 3.2 — Session Types at a Glance

A quick-reference field guide to the species of sessions you'll encounter in the wild:

| Session Type        | Description                                                                                                                                                                                                                                                                        |
|---------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Individual Therapy  | One-on-one with a client. The bread and butter. 16-37 minutes, 38-53 minutes, or 54+ minutes, <b>the higher the time the more credit; the shorter the time, the more documentation</b> . This is where the deep work happens — and where you'll feel most like a "real therapist." |
| Family Therapy      | Session with the client and family member(s). Session minimum is 26 minutes. Can be in-person at the school, by tele-health, or a home visit. Coordination-heavy but clinically rich. Scheduling these requires patience and persistence.                                          |
| Group Therapy       | Structured therapeutic group with 2+ clients. Requires a curriculum, clear goals, and documentation for each participant. Efficient and impactful; and yes, you have to write a note for EACH kid, but group notes have a shared component and take less time.                     |
| Crisis Intervention | Unplanned response to a client in acute distress. Document thoroughly: timeline, interventions, outcome, follow-up plan. These sessions are exhausting. Plan recovery time after.                                                                                                  |
| Intake assessment   | The intake session covers the essentials: initial evaluation, age-appropriate                                                                                                                                                                                                      |



|  |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                               |
|--|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
|  | <p>assessments tied to the client’s primary diagnosis, consent to treat, HIPAA and privacy limitations, and any other required forms. You’ll also begin treatment planning and set collaborative goals. This is the “let’s get oriented” moment, not a paperwork scavenger hunt.</p> <p>For minors, you meet with both the parent and the child on the day of intake. No exceptions, no “I’ll catch them next week.” You must enter a diagnosis before you close out the brief note. All intake documents and any additional assessments are completed the same day.</p> <p>Billing a service without documentation doesn’t make you look busy — it makes you look fraudulent, and compliance will absolutely notice. Think of it this way: if you wouldn’t want to explain it in a meeting, don’t do it.</p> |
|--|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|

## 3.3 — Protecting Your Energy Budget

You have a finite amount of clinical energy each day. That's not weakness, it's biology. Empathy is metabolically expensive. Active listening is cognitively demanding. Holding space for a child's pain while maintaining professional boundaries while remembering to document while watching the clock? That takes everything you've got. The therapists who last in this field aren't the ones who give everything every day. They're the ones who learn to budget.

### Energy Budget Worksheet

*Map your typical day's energy expenditure. Be honest, this isn't a graded assignment, it's a tool to set you up for success.*

| Activity | Energy Cost (Low / Med / High) | Can I Adjust This? |
|----------|--------------------------------|--------------------|
|          |                                |                    |
|          |                                |                    |
|          |                                |                    |
|          |                                |                    |
|          |                                |                    |

### Energy Recovery Strategies

- **Front-load intensity.** Stack your hardest sessions earlier when your tank is fullest.
- **Build buffer zones.** 10-minute gaps between intense sessions aren't luxury; they're infrastructure.
- **Use transitions intentionally.** Walk, hydrate, decompress. The hallway between sessions is a regulation tool.
- **Know your redline.** The point where you're *performing* therapy, not *doing* it. If you can feel yourself acting empathetic instead of being empathetic, you've crossed it.
- **Debrief heavy sessions in supervision,** not in your head on the drive home. Your car is not a consultation room.



# The Energy Cycle ⚡



## Understanding Productivity in the Ecosystem

In this ecosystem, your "fuel" is your time and energy. Let's decode how to use it wisely.

### Credit Counts: Understanding Our Energy Budget

#### ENERGY IN → ENERGY OUT -

- Charging Station
- Direct Sessions
- Group Work
- Family Consults

- Energy Output
- Documentation
- Collaboration
- Admin Tasks

Sustainable & Balanced →



### Session Types: What Powers Up?



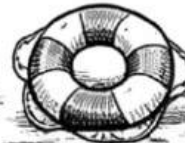
Individual Therapy



Family Sessions



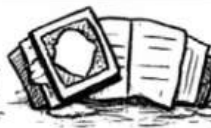
Group Therapy



Crisis Support



Collateral Contacts



Assessments



### The Energy Tracker: What's Your Current Charge?

#### FULL POWER!

In the Green Zone

#### KEEPING PACE

In the Yellow Zone

#### RUNNING LOW

In the Red Zone

⚡ Energy Boosters: What charges you up? \_\_\_\_\_

⌚ Energy Drainers: What depletes your fuel? \_\_\_\_\_

🔋 Recharge Plan: One step I'll take to refuel: \_\_\_\_\_

# Section 4: Leaving a Trail

*"In the field, you document everything — not because someone's checking, but because the trail you leave is how others find their way. Your notes are the trail."*

## 4.1 — The Documentation Trail Map

Documentation is the least glamorous and most non-negotiable part of this work. It protects your clients, protects you, ensures continuity of care, and keeps the entire system accountable. It's also the thing most new therapists fall behind on first. This section is about building a documentation rhythm that's sustainable — not perfect, but consistent.

### THE TRAIL: Documentation Flow

**Session Happens (Eleos takes notes) → 10 minutes before session end stop Eleos, → Note Drafted or Completed** while in session (client completing collaborative documentation worksheet while therapist writes notes, compare → **Note Submitted** in system → **Done**. Move on.

### THE GOLDEN RULE

**If it wasn't documented, it didn't happen.**

That's not a threat — it's a legal and clinical reality. Your note is the evidence that a service was provided, that clinical judgment was exercised, and that a plan exists. If you ever end up in a situation where someone asks, "What happened in that session?" your note is the answer. Not your memory. Not your good intentions. Your note.

## 4.2 — Finding Your Note-Writing Rhythm

Some therapists write notes between sessions. Some batch them at the end of the day. Some use their admin blocks. The method matters less than the consistency. Here are the most common approaches — find the one that fits your brain.

### THE IN-SESSION SCRIBE

You knock out a quick draft as the final intervention as the last 10-15 minutes of session using a combination of Eleos and collaborative documentation.

**Pro:** Accuracy and recall are high. You capture the nuance — the exact phrases the kid used, session theme. This approach requires less admin time after session, which means more time for kids OR finishing up early!

**Con:** Requires a stable internet signal and functioning technology.

### THE END-OF-DAY BATCHER

You see all your clients, then sit down and write all your notes in one focused block.

**Pro:** Efficient, gets you in a writing flow.

**Con:** Details blur by session 4, and if your day runs long, notes get pushed to in the evenings to meet the 24-hour mark. If this is your preferred method, the use of Eleos as a scribe helps to ensure validity and fidelity of your progress notes.

### THE ADMIN BLOCK ARCHITECT

You protect specific blocks on your schedule for documentation and treat them like client appointments.

**Pro:** Sustainable, boundaried, and dependable.

**Con:** Reduces available time for client care, which is our primary responsibility. If this is your preferred method, make sure to prioritize sessions and monitor your target throughout the day, but wouldn't you rather spend this time having lunch, protecting this as a cherished self-care break, or getting to leave right before dismissal to avoid the car rider lane?

#### THE 24-HOUR COMPASS

Whichever method you choose: **Notes are to be completed within 24 hours of the session.** If you're consistently falling behind, that's a systems issue — bring it to supervision before it becomes a pattern. Your supervisor would rather help you fix your schedule than find out you're three weeks behind on documentation.

## 4.3 — Note Structure Quick Reference

Centerstone has specific documentation requirements. This is a general-purpose reference for the most common note structures used in school-based settings. Keep this page bookmarked — you'll come back to it.

### Format

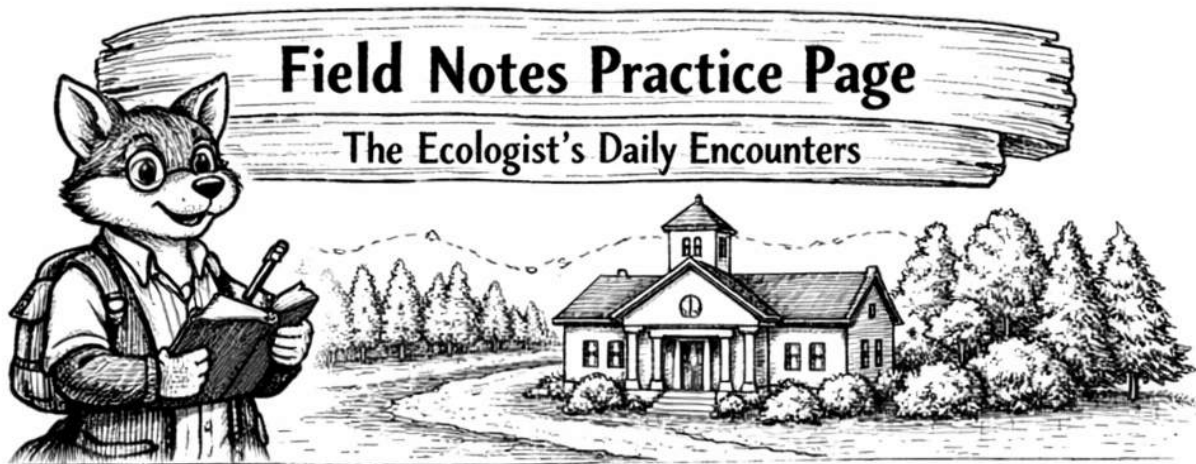
| Section                 | To Include                                                                                                                                                                                                                                  |
|-------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Session goal/objective: | What was the identified objective for the session (pulled from the active care plan.) Only chose one.                                                                                                                                       |
| Progress:               | This is DIRECTLY related to the session goal and speaks to the effectiveness of interventions/session, not a general overview of client well being.                                                                                         |
| Life occurrences:       | Includes any event that will affect client progress. Starting your session with a check-in will typically yield this data and guide your intervention. Why not lead with having your client share the sunny and stormy parts of their week. |

|                                 |                                                                                                                                                                                                                                           |
|---------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Intervention & Client response: | This section should be like a volleyball game. Each intervention therapist serves across the net the client's response (or reaction) is the runt that tells therapist where to aim next. Focus on this and avoid too many client reports. |
| Obstacles:                      | What barriers were present that made it difficult for therapy or intervention to occur.                                                                                                                                                   |
| Plan:                           | What is the starting point for the next session, when will you meet, will you need assessments, and what homework are you assigning (was the homework communicated to parent?)                                                            |

## REMINDERS

- Part of good clinical documentation is using appropriate assessments based on client age and primary dx.
- Clinical documentation isn't just session notes. It includes memos to chart, care plans, and client paperwork. Our electronic health record (Avatar) is medical documentation, and you are an outpatient therapist. Keep your documentation clear and concise, clinical, and professional — your words matter, and they help ensure quality care for your clients.
- The note should speak to good and bad progress. To justify continued care, therapist's documentation should speak to the client need including if there is a regression or increased behavior concerns (which are not a reflection on the quality of care a client receives).
- Care plans in draft longer than 7 days are subject to deletion by TL or Clinical Supervisor, without warning.
- Be proactive and use provided tools to update paperwork in the 30-day window prior to it expiring which will reduce your stress.
- Be prepared to be creative with parent engagement to update paperwork. Whether they work an off schedule, have had a negative experience with another provider, or are at their wits end, it is highly unlikely they are being resistance, so remember to be consistent and compassionate with your approach, but also be boundaried. If a parent is non-responsive and you have documented appropriately, staff with your TL, there will be clients that will need to be discharged, we just want to do it in a way that lets a parent know that when or if they are ready, we will be there for them.





# Field Notes Practice Page

The Ecologist's Daily Encounters

## Intervention & Client Response

Write an "intervention & client response" section of a progress note for a 20-minute individual session.

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## Obstacles

Write an "obstacles" section for a chaotic family session via telehealth.



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## Progress Toward the Goal

You had a tough session with a 6-year-old who destroyed your office. You had great interventions but nothing landed. The identified goal for the session was "client will wait 5 seconds before responding in 4 out of 5 opportunities."

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## Leaving a Trail

Charting the School Landscape

Documenting  
Success!



Every habitat has unique landmarks. Let's map out your surroundings.

### The Path to Great Notes

Write clear and focused progress notes to track your journey.



**Session Goal:** What's the aim of today's session?



**Progress:** Steps taken and milestones reached.



**Life Occurrences:** Notable client events impacting therapy.



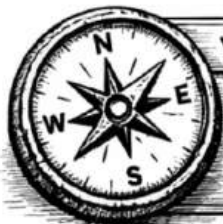
**Interventions & Client Response:** Techniques used and client reactions.



**Obstacles:** Challenges encountered along the way.



**Plan:** Next steps for the path ahead.



### Your Note-Taking Compass



**Use Eleos**

Record themes & interventions  
Stay present in the session

### 24-Hour Deadline



Notes must be completed within 24 hours

### Note-Taking Trail

**Session Goal:**

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**Progress:**

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**Life Occurrences:**

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**Interventions & Response:**

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**Obstacles:**

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**Plan:**

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Draw your feelings

— How I felt at the beginning of session — — How I am feeling now —



— One thing we discussed in session that was important to me, or helpful was... —

— This week I will practice \_\_\_\_\_ with my parent and I'll share when I come in again.



### Prep for Next Time...



When we meet again, we can talk about \_\_\_\_\_

I would really like if we could also do \_\_\_\_\_



My next session is on \_\_\_\_\_

### Notes to My Parent:

# Section 5: Reading the Habitat

*"You can't tend an ecosystem you don't understand. Before you intervene, observe. Before you change anything, learn what's already growing."*

## 5.1 — Navigating School Culture

Every school has a personality. Some buildings are warm, chaotic, and loud. Others are rigid, quiet, and buttoned-up. Most are somewhere in between, with microclimates in every hallway. The third-grade wing might be a tropical rainforest of creativity while the front office operates like a military base. Your job in the first 30 days is not to change anything. It's to observe. To listen. To learn the unwritten rules, the ones no one puts in the handbook, but everyone follows.

### THE FIRST 30 DAYS: An Observer's Checklist

- ☐ Learn every staff member's name you interact with. Use them. Repeatedly.
- ☐ Ask the front office how they prefer you to sign in/out, where to park, where you can work
- ☐ Eat lunch in the staff lounge at least once. Bring something to share if you can.
- ☐ Ask a veteran teacher: "What should I know about this building that nobody will think to tell me?"
- ☐ Learn the school's discipline system and how it works in practice (not just on paper)
- ☐ Identify the "cultural brokers" — the staff members everyone trusts and gravitates toward
- ☐ Notice what the school celebrates and what it avoids discussing
- ☐ Observe how crises are handled before you assume your role in them
- ☐ Learn the schools safety procedures (fire drills, tornado drills, intruder drills)
- ☐ Introduce yourself to the custodial staff. They are essential humans in this ecosystem.
- ☐ Find the bathroom. Seriously. Find more than one. Know which ones are quiet enough for a go-second reset.

## 5.2 — Confidentiality in a Fishbowl

You are a clinical professional operating in a building designed for education, not therapy. There are no soundproof offices. Teachers will ask "how did it go?" after a session. The guidance counselor will want updates. A well-meaning aide will say "I heard Johnny had a rough session — is he okay?" You need scripts for this. Because "I can't tell you that" is technically accurate but relationally clunky. Here's how to hold the boundary while keeping the relationship.

**SCENARIO 1: A teacher asks, "What's going on with [student]? They've been so off lately."**

*"I appreciate you noticing that and flagging it. I can't share details about what we talk about, but I can tell you I'm working with them. If you're seeing specific behaviors in class, I'd love for you to share those with me — it helps me support them better."*

**SCENARIO 2: An administrator asks for a "quick update" on a student's therapy.**

*"I'm happy to share that [student] is actively engaged in services. For anything clinical, I'd want to loop in their guardian and my supervisor so we're all on the same page. Can I set that up?"*

**SCENARIO 3: A parent asks in the hallway, with other parents nearby, about their child's session.**

*"I'd love to talk with you about how things are going! Can I call you later today when we can have a real conversation? I want to give you my full attention — and you deserve more than a hallway chat."*

**SCENARIO 4: Another therapist or school counselor asks to share notes or discuss a mutual student.**

*"I want to collaborate — let me check that I have the right consents in place and we'll set up a time to talk. I'd rather do it right than do it fast."*

## 5.3 — Crisis in the Building: Know Your Lane

Crises happen in schools. A student discloses abuse. A fight breaks out. A lockdown is called. A child has a panic attack in the hallway. Your instinct will be to help, and you should. But you also need to know exactly what's yours and what belongs to the school's crisis protocol. Freelancing in a crisis helps nobody and can actually make things worse.



### **ASK YOURSELF THESE THREE QUESTIONS:**

**1. "Is this student on my active caseload?"**

If yes, you have a clinical role. Respond accordingly. You know this student. You have context. Step in.

**2. "Is there an immediate safety concern (harm to self or others)?"**

If yes, follow your Centerstone's crisis protocol AND the school's safety procedures simultaneously. Document everything. Time, place, what was said, what you did, who you called.

**3. "Is this a school-level crisis (lockdown, building emergency, fight) that doesn't involve my caseload?"**

Follow the school's instructions. Support where asked. Do not freelance. Debrief with your supervisor afterward.



**THE HARDEST PART:** Sometimes the most therapeutic thing you can do in a crisis is stay calm, stay out of the way, and be available after the dust settles. Not every crisis needs you at the center. Your presence and steadiness ARE the intervention. A calm adult in a chaotic building is more valuable than you know.



## Reading the Habitat



Observing Your School Environment

Time to investigate!

Be alert and aware of your surroundings. Let's take a closer look!

### Habitat Clues: What to Observe

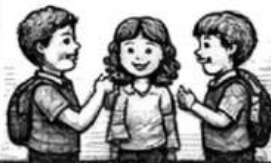
#### School Climate

Mood & Atmosphere



#### Group Dynamics

Social Interactions



#### Rules & Routines

Daily Expectations



### Field Notes: Recording Observations



What I Noticed:



Conversations I Heard:



Questions to Explore:

### Investigator's Tips: Tools for Discovery



Use All Your Senses



Watch, listen & observe carefully.



Stay Curious

Ask "Why?" and "How?"



Blend In

Be calm & unobtrusive.



# Section 6: Shelter and Sustenance

*"Even the most dedicated field researcher builds a shelter. Even the strongest swimmer comes up for air. Sustaining yourself isn't a reward for hard work — it's a prerequisite for it."*

## 6.1 — Building Your Shelter: Boundaries That Protect Without Isolating

Boundaries in this work aren't walls. They're shelters. Structures that protect you from the elements while still allowing you to engage with the world outside. The goal isn't to become unreachable. It's to become sustainable. A collapsed therapist helps no one. A boundaried therapist helps everyone on their caseload for years to come.

### Boundary Inventory

*For each category, reflect honestly on where you tend to over-extend and where you hold firm. There's no grade. This is reconnaissance.*

**TIME BOUNDARIES:** "Do I leave on time? Do I check emails after hours? Do I let sessions run long?"

**EMOTIONAL BOUNDARIES:** "Am I carrying my clients' stories home? Do I feel personally responsible for their outcomes?"

**ROLE BOUNDARIES:** "Am I doing things that aren't my job because no one else will? Am I saying yes to tasks that pull me from clinical work?"

**RELATIONAL BOUNDARIES:** "Am I trying to be liked by school staff at the expense of my clinical integrity? Am I over-sharing about my own life?"

**PHYSICAL BOUNDARIES:** "Am I eating? Moving? Hydrating? Using the bathroom?" (Yes, this needs to be on the list.)

## 6.2 — The Art of the Compassionate No

You will be asked to do things that aren't part of your role in the school's ecosystem. Not because people are trying to take advantage of you — but because you're helpful, you're present, and you clearly care. The ability to say no with warmth and clarity is one of the most important clinical skills you will ever develop. It's also one of the hardest. Here are scripts that keep the relationship intact while protecting your role.



### SCRIPTS THAT KEEP THE RELATIONSHIP INTACT

- *"I wish I could help with that — it sounds important. That's really [counselor/admin/teacher]'s area, though. Want me to help you connect with them?"*

- *"I have to protect my session time today, but I want to make sure this gets handled. Can we find a time to talk about it tomorrow?"*
- *"I appreciate you thinking of me for that. Right now, my caseload needs my full bandwidth. Let me suggest someone who might be a better fit."*
- *"That's outside my scope, but I don't want to leave you hanging. Here's what I CAN do..."*
- *"I'm going to say no to this one — and I want you to know it's not because I don't care. It's because I care about doing my actual job well, and I can't do that if I'm stretched into everything."*

## 6.3 — Weather Station: Your Internal Forecast

Compassion fatigue, vicarious trauma, and burnout don't announce themselves. They accumulate — like weather systems building on the horizon. You don't wake up one day burned out. You erode. This check-in is a barometer. Use it monthly, or whenever the forecast feels off.

### Internal Weather Assessment

*Rate each statement honestly. Not how you SHOULD feel. How you DO feel.*

**Scale:** 1 = Not at all | 2 = Sometimes | 3 = Often | 4 = Almost always

| Statement                                                        | Rating (1–4) |
|------------------------------------------------------------------|--------------|
| I dread going to work or to a specific school                    |              |
| I have trouble sleeping, or I'm sleeping too much                |              |
| I feel emotionally numb or disconnected from my clients          |              |
| I'm more irritable with coworkers, family, or friends than usual |              |
| I'm falling behind on documentation and can't seem to catch up   |              |



|                                                                              |  |
|------------------------------------------------------------------------------|--|
| I feel guilty when I'm NOT working                                           |  |
| I've stopped doing things that used to recharge me                           |  |
| I catch myself thinking "what's the point?" about this work                  |  |
| I'm isolating — from my team, my supervisor, my people                       |  |
| I'm having physical symptoms (headaches, stomach issues) with no clear cause |  |
| <b>TOTAL:</b>                                                                |  |

### Scoring Guide

**10–15: Clear skies** ☀️ — "You're managing well. Keep doing what you're doing and stay aware. Good weather doesn't last forever, so tend the habits that got you here."

**16–25: Partly cloudy** ⛅️ — "Some weather moving in. Check in with your supervisor. Adjust something before it builds. This is the intervention window."

**26–34: Overcast** ☁️ — "The system is stressed. This isn't sustainable. Bring this to supervision this week. Consider what needs to change — in your schedule, your caseload, or your support system."

**35–40: Storm warning** ⚡️ — "You need support now. Talk to your supervisor, your program manager, or someone you trust. This isn't a character flaw — it's a signal. Signals are there to keep you safe"

## 6.4 — The Sustenance Matrix

Self-care isn't bubble baths and motivational quotes. (Unless those work for you — genuinely, no judgment.) Real self-care is about identifying what ACTUALLY restores you and making sure it's happening regularly, not just when you're already depleted. Think of this as a nutritional plan for your whole self.

### The Sustenance Matrix

| PHYSICAL SUSTENANCE                                                                                                                                                               | EMOTIONAL SUSTENANCE                                                                                                                                      |
|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------|
| <i>What does your body need?</i><br><i>Movement, rest, nutrition, fresh air, sleep?</i><br>When was the last time I actually did this?                                            | <i>What feeds your heart?</i><br><i>Connection, laughter, creative expression, a good cry, time alone?</i><br>When was the last time I actually did this? |
| PROFESSIONAL SUSTENANCE                                                                                                                                                           | RELATIONAL SUSTENANCE                                                                                                                                     |
| <i>What keeps you sharp and engaged?</i><br><i>Training, reading, peer consultation, mentorship, trying new interventions?</i><br><br>When was the last time I actually did this? | <i>Who fills your cup?</i><br><i>Friends, family, partner, community, your therapy team?</i><br><br>When was the last time I actually did this?           |



## Shelter & Sustenance



Finding Your Refuge in the School

Let's gather what we need!

Let's gather what we need!

### Essentials for Survival

#### Healthy Snacks

Fuel for the day



#### Recharge Spots

Places to Rest



#### Supply Stash

Tools and Materials



#### Safe Spaces

Areas for Comfort



### Gathering Supplies: Find Your Resources



Healthy Options I can find:



Relaxing Areas to go to:



Helpful Materials I can use:

### Survival Strategies: Tips for Self-Care



#### Take a Break

Rest and recharge when needed.



#### Stay Nourished

Eat healthy & stay hydrated.



#### Seek Sanctuary

Find a quiet place to unwind.



# Section 7: Field Notes

*"The best field researchers don't just collect data. They reflect on what they're learning — about the ecosystem, and about themselves."*

## 7.1 — Weekly Field Observations

*Reproduce this page weekly. Keep them. You'll be amazed what you learn about your own patterns over time.*

 **FIELD NOTES — WEEK OF:** \_\_\_\_\_

### **WHAT I OBSERVED THIS WEEK:**

*What happened? What patterns did I notice? What surprised me?*

### **WHAT'S GROWING:**

*What's working? Where am I seeing progress — in my clients, in myself, in my systems?*

### **WHAT FELT HEAVY:**

*What was hard? What weighed on me? What do I want to process?*

### **WHAT I WANT TO BRING TO SUPERVISION:**

*Questions, cases, skill areas, things I'm unsure about*

### **ONE THING I DID WELL THIS WEEK:**

*Name it. You need practice receiving this.*

## 7.2 — Monthly Expedition Report

**MONTHLY CHECK-IN — MONTH:** \_\_\_\_\_

### **THE WEATHER THIS MONTH:**

*Circle one and add context:*

-  **Sunny** — I felt grounded, effective, and connected
-  **Mostly clear** — Good overall, a few cloudy patches
-  **Cloudy** — I got through it, but I'm tired
-  **Rainy** — It was a hard month. I need to talk about it.
-  **Stormy** — I'm struggling and I need support.

Context:

### **WHAT THE DATA SAYS:**

*How's my productivity? Am I on track with documentation? What do the numbers look like?*

**WHERE I GREW THIS MONTH:**

*A skill I developed, a relationship I built, a hard moment I handled well*

**WHAT NEEDS ADJUSTING:**

*Schedule tweaks, approach shifts, conversations I need to have*

**WHAT I WANT MY SUPERVISOR TO KNOW:**

*Things I might not say out loud unless prompted*

## 7.3 — Supervision Preparation Page

*Reproduce this for every supervision session. Coming prepared is a gift to yourself and your supervisor.*

**SUPERVISION PREP — DATE:** \_\_\_\_\_

**Cases to Discuss**

| Client (initials) | Issue / Question |
|-------------------|------------------|
|                   |                  |
|                   |                  |
|                   |                  |

**Skill Areas I Want to Grow**

**Something I'm Proud Of:**

**Something I'm Struggling With:**

**Questions for My Supervisor:**

**Administrative Items:**

## 7.4 — Quarterly Terrain Review

### QUARTER \_\_\_\_\_ REVIEW — THE LONG VIEW

When I started this quarter, I was hoping to...

Looking back, what actually happened was...

The three most important things I learned this quarter:

- 1.
- 2.
- 3.

If I could go back and tell beginning-of-quarter me one thing, it would be:

Next quarter, I want to focus on:

Next quarter, I want to let go of:

One thing I want to celebrate about this quarter before I move on:

# Field Notes

Reflecting on the Journey

Time to reflect!

## Capturing Your Thoughts

Today's Highlights

What I Learned...

What I Learned...

How I'm Feeling...

## Deep Reflections



What went well in my sessions?



What challenges did I encounter?



How will I grow from this experience?

## Trail Wisdom: Insights for the Path Ahead



Pause & Ponder

Take time to think it through.



Share the Story

Reflect & consult with others.



Set Intentions

Plan for the trail ahead.



# Section 8: Base Camp Essentials

*"Every field researcher keeps a base camp stocked with essentials. These are yours."*

## 8.1 — Jargon Decoder

School-based mental health has its own language. Some of it comes from education. Some from clinical practice. Some from billing. All of it will be thrown at you in meetings like everyone assumes you already know it. You don't have to memorize this page — just know it's here.

| Acronym | What It Means                                                                                                        |
|---------|----------------------------------------------------------------------------------------------------------------------|
| IEP     | Individualized Education Program — a legally binding document outlining special education services and goals         |
| 504     | Section 504 accommodation plan — provides accommodations for students with disabilities who don't qualify for an IEP |
| BIP     | Behavior Intervention Plan — a plan for addressing challenging behaviors based on an FBA                             |
| FBA     | Functional Behavior Assessment — an analysis of a student's behavior to determine its function                       |
| MTSS    | Multi-Tiered System of Supports — a framework for providing tiered levels of support to students                     |
| SST     | Student Support Team — a school-based team that reviews students who need additional support                         |

|                |                                                                                                                                                                                   |
|----------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>LEA</b>     | Local Education Agency — typically the school district                                                                                                                            |
| <b>HIPAA</b>   | Health Insurance Portability and Accountability Act — federal health information privacy law                                                                                      |
| <b>FERPA</b>   | Family Educational Rights and Privacy Act — federal law protecting student education records                                                                                      |
| <b>MBC</b>     | Measurement-based care- a collaborative and consistent use of assessments with therapist, parents, and schools as an intervention, track progress, and inform treatment planning. |
| <b>Psc-17</b>  | Pediatric symptoms checklist- 17 question parent report screen administered monthly                                                                                               |
| <b>CPT</b>     | Current Procedural Terminology — the billing codes used for clinical services                                                                                                     |
| <b>ROI</b>     | Release of Information — written consent to share clinical information with a specified party                                                                                     |
| <b>TCP</b>     | The Care Plan — the document outlining clinical goals, interventions, and timelines                                                                                               |
| <b>SI / HI</b> | Suicidal Ideation / Homicidal Ideation — clinical terms for thoughts of self-harm or harm to others                                                                               |
| <b>PHQ</b>     | Patient Health Questionnaire — a standardized screening tool for depression                                                                                                       |

|                |                                                                                                   |
|----------------|---------------------------------------------------------------------------------------------------|
| <b>CSSRS</b>   | Columbia Suicide Severity Rating Scale — a standardized suicide risk assessment tool              |
| <b>EBP</b>     | Evidence-Based Practice — interventions supported by clinical research                            |
| <b>SUD</b>     | Substance Use Disorder — clinical diagnosis related to substance misuse                           |
| <b>ACE</b>     | Adverse Childhood Experience — categories of childhood trauma linked to long-term health outcomes |
| <b>DX / BX</b> | Approved shorthand for diagnosis; approved shorthand for behavior                                 |

## 8.2 — Emergency Protocols Quick Card

### WHEN THINGS GET SERIOUS — KNOW YOUR PROTOCOLS

This page should be accessible at all times. Dog-ear it. Photograph it. Tape it to your clipboard. When a crisis happens, your brain will not calmly recall training — it will reach for whatever is closest.

#### IF A STUDENT DISCLOSES ABUSE OR NEGLECT

- **You are a mandated reporter. This is not optional.**
- Document the disclosure using the student's own words — do not paraphrase, do not interpret
- Report to your state's child protective services hotline
- Notify your supervisor immediately
- **Do NOT investigate. Do NOT interview the child further. Do NOT notify the alleged perpetrator.**
- After completing the DCS report (same day); complete the appropriate RL6 incident report
- Document everything: what was said, when, what you did, who you called, timestamps

#### IF A STUDENT EXPRESSES SUICIDAL IDEATION

- **Do not leave the student alone**

- Conduct a risk assessment starting with the CSSRS.
- Follow their pathway enrollment protocol and update your clinical supervisor and the parent when client is enrolled in a prevention pathway.
- Document the assessment, your clinical judgment, and the Stanley Brown safety plan; provide parent the safety sweep checklist.
- Explain the client's commitments for pathway enrollment and therapist's commitments.
- Follow up. Then follow up again. Then follow up one more time.

### **IF A STUDENT IS IN ACUTE CRISIS (aggression, panic, dissociation)**

- **Ensure physical safety first — yours and the student's**
- Use verbal de-escalation strategies appropriate to the situation
- Notify school staff if additional support is needed
- Do not physically restrain unless trained and authorized
- Document the incident, your interventions, and the outcome
- Debrief with your supervisor

### **CRITICAL NUMBERS — Fill These in During Onboarding**

**Help Desk (IT support) phone:**

**Hours of Operation:**

**Local emergency room phone:**

**Address:**

**Agency crisis line:**

**State child abuse hotline:**

**Suicide & Crisis Lifeline: 988**

**Local emergency: 911**

**School #1:**

**Hours:**

**School #2:**

**Hours:**

## 8.3 — Your Key Contacts Directory

Fill this in during your first week. Update it as your ecosystem grows. Having this information in one place will save you on the days when you need it most.

### Key Contacts

#### MY TEAM LEAD (ADMINISTRATIVE SUPERVISOR)

Name:

Phone:

Email:

#### MY CLINICAL SUPERVISOR

Name:

Phone:

Email:

#### PROGRAM MANAGER

Name: Topher Fuqua, LMFT

Phone: 615-674-0629

Email: Christopher.fuqua@centerstone.org

#### SCHOOL CONTACT (School 1)

Name:

Role:

Phone:

#### SCHOOL CONTACT (School 2)

Name:

Role:

Phone:

**ASSIGNED CLINIC**

Phone:

Address:

Hours:

Clinic Manager:

Office Manager:

**IT SUPPORT**

Phone:

Email:

**HR / BENEFITS**

Phone:

Email:

**PEER SUPPORT / MENTOR**

Name:

Phone:



# Base Camp Essentials

The  
Your Toolkit for School Survival

Equip yourself  
for success!



## Essential Gear: Tools of the Trade

### Session Notes

Notebook & Pen



### Therapy Apps

Digital Aids



### Reference Guides

Helpful Manuals



### Contact List

Key Connections



## Pack Your Kit: Prepared & Ready



Things I Need in My Bag:



Daily Reminders:



Emergency Info to Keep Handy:

## Survival Tips: Wisdom from the Owl



### Stay Organized

Keep everything in it's place.



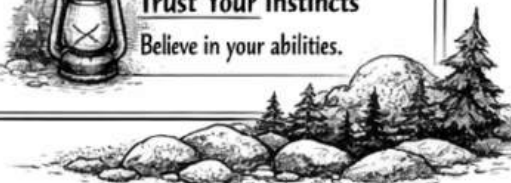
### Light the Way

Be a guiding light for others.



### Trust Your Instincts

Believe in your abilities.





# A Note for the Trail Ahead

If you made it this far in the guide, you're already showing up with intention. That counts. More than you know, that counts.

There will be days when the work feels impossible — when three sessions cancel, a crisis derails your afternoon, your documentation piles up, and a student you've been pouring into takes two steps backward. Those days are real.

And there will be days when a student says something in session that makes you understand, deep in your bones, why you chose this work. When a teacher stops you in the hallway to say "whatever you're doing, keep doing it." When a parent cries on the phone because for the first time, someone is helping. Those days are real too.

You don't have to do this alone. That's the whole point of a team, a supervisor, and a field guide. The ecosystem is bigger than you, and that's not a limitation — it's a relief. You were never supposed to carry all of it.

Tend the work. Tend yourself. And when the terrain gets rough — because it will — look back at how far you've already come. You'll find that the trail behind you is longer than you thought. That you've learned more than you realize. That you're more ready than you feel.

**See you in the field.**